

WARNING: According to FDA, this warning applies to all antipsychotic drugs (conventional and atypical). These drugs are not indicated in the treatment of dementia-related psychosis. This is because their use is associated with increased risk of mortality in elderly patients treated for dementia-related psychosis.

This drug includes: Compazine (prochlorperazine), Haldol (haloperidol), Loxitane (loxapine), Mellaril (thioridazine), Molan (molsidolam), Navane (thioridazine), Olanzapine, Prolixin (perphenazine), Seroquel (quetiapine), Thorazine (chlorpromazine), Trilafon (perphenazine), Zyprexa (olanzapine).

Use in specific population / pregnancy (non-teratogenic effects): Neonates exposed to antipsychotic drugs, during the third trimester of pregnancy are at risk for extrapyramidal and/or withdrawal symptoms following delivery. There have been reports of agitation, hyperreflexia, hypotonia, tremor, somnolence, respiratory distress and feeding disorders in these neonates. These symptoms have been associated with the use of antipsychotic drugs during pregnancy. Therefore, the use of antipsychotic drugs during pregnancy should be carefully monitored. These products should be used during pregnancy only if the potential benefit justifies the potential risk to the fetus. There is a probability of death in elderly patients who suffer from dementia-related psychosis.

Inactive Ingredients: Sodium lauryl sulphate, maize starch, microcrystalline cellulose, crosscarmellose sodium, colloidal silicon dioxide, magensium stearate and FD & C blue NO2.

PHARMACEUTICAL FORM: Tablets.

POSODOLOGY: • **Adults:** **Schizophrenia:** The recommended starting dose for Apilipex is 10 or 15 mg/day with a maintenance dose of 15 mg/day administered on a once-a-day schedule without regard to meals. Apilipex is effective in a dose range of 10 to 30 mg/day. Enhanced efficacy at doses higher than a daily dose of 15 mg has not been demonstrated although individual patients may benefit from a higher dose. The maximum daily dose should not exceed 30 mg.

Manic episodes: The recommended starting dose for Aripiprex is 15 mg administered on a once a day schedule without regard to meals as monotherapy or combination therapy. Some patients may benefit from a higher dose. The maximum daily dose should not exceed 30 mg. Recurrence prevention of manic episodes in bipolar I disorder: For preventing recurrence of manic episodes in patients who have been receiving aripiprazole, continue therapy at the same dose. Adjustments of daily dosage, including dose reduction should be considered on the basis of clinical status.

Pediatric population: *Schizophrenia in adolescents 15 years and older:* The recommended dose for Aripipex is 10 mg/day administered on a once a day schedule without regard to meals. Treatment should be initiated at 2 mg for 2 days, titrated to 5 mg for 2 additional days to reach the recommended daily dose of 10 mg. When appropriate, subsequent dose increases should be administered in 5 mg increments without exceeding the maximum daily dose of 30 mg. Aripipex is effective in a dose range of 10 to 30 mg/day. Enhanced efficacy at doses higher than a daily dose of 10 mg has not been

• **Patients with hepatic impairment:** No dosage adjustment is required for patients with mild to moderate hepatic impairment. In patients with severe hepatic impairment, the data available are insufficient to establish recommendations. In these patients dosing should be managed cautiously. However, the maximum daily dose of 30

Elderly: The effectiveness of Apilipex in the treatment of schizophrenia and bipolar I disorder in patients 65 years of age or older has not been established. Owing to the greater sensitivity of this population, a lower starting dose should be considered when clinical factors warrant.

• **Dose adjustments due to interactions:** When concomitant administration of potent CYP3A4 or CYP2D6 inhibitors with aripiprazole occurs, the aripiprazole dose should be reduced. When the CYP3A4 or CYP2D6 inhibitor is withdrawn from the combination therapy, aripiprazole dose should then be increased. When concomitant administration of potent CYP3A4 inducers with aripiprazole

CONTRAINDICATIONS: Hypersensitivity to the active substance or to any of the excipients.

Cardiovascular disorders: Aniprazole should be used with caution in patients with known cardiovascular disease (history of myocardial infarction or ischaemic heart disease, heart failure or conduction abnormalities), cerebrovascular disease, conditions which would predispose patients to hypotension (dehydration, hypovolemia, and treatment with antihypertensive medicinal products) or hypertension, including accelerated or malignant. Cases of venous thromboembolism (VTE) have been reported with antipsychotic drugs. Since patients treated with antipsychotics often present with acquired risk factors for VTE, all possible risk factors for VTE should be identified before and during treatment with Apilixeb and preventive measures undertaken.

Tardive dyskinesia: In clinical trials of one year or less duration, there were uncommon reports of treatment emergent dyskinesia during treatment with aripiprazole. If signs and symptoms of tardive dyskinesia appear in a patient on aripiprazole, dose reduction or discontinuation should be considered. These symptoms can temporarily deteriorate or can even arise after discontinuation of treatment.

or blood pressure, tachycardia, diaphoresis and cardiac dysrhythmia). Additional signs may include elevated creatine phosphokinase, myoglobinuria (rhabdomyolysis), and acute renal failure. However, elevated creatine phosphokinase and rhabdomyolysis, not necessarily in association with NMS, have also been reported. If a patient develops signs and symptoms indicative of NMS, or presents with unexplained high fever without additional clinical manifestations of NMS, all antipsychotic medicinal products, including Apilixen, must be discontinued.

Elderly patients with dementia-related psychosis: Increased mortality: In three placebo-controlled trials (n= 938; mean age: 82.4 years; range: 56-99 years) of aripiprazole in elderly patients with psychosis associated with Alzheimer's disease, patients treated with aripiprazole were at increased risk of death compared to

Cerebrovascular adverse reactions: In the same trials, cerebrovascular adverse reactions (e.g. stroke, transient ischaemic attack), including fatalities, were reported in patients (mean age: 84 years; range: 78–88 years). Overall, 1.3% of aripiprazole-treated patients reported cerebrovascular adverse reactions compared with 0.6% of placebo-treated patients in these trials.

Hyperglycemia and diabetes mellitus. Hyperglycemia, in some cases extreme and associated with ketoacidosis or hyperosmolar coma or death, has been reported in patients treated with atypical antipsychotic agents, including Apilipex. Risk factors that may predispose patients to severe complications include obesity and family history of diabetes. In clinical trials with aripiprazole, there were no significant differences in the incidence rates of hyperglycemia-related adverse reactions (including diabetes) or in abnormal glycemia laboratory

Hypersensitivity: As with other medicinal products, hypersensitivity reactions, characterised by allergic symptoms, may occur with aripiprazole.

Dysphagia: Oesophageal dysmotility and aspiration have been associated with antipsychotic treatment, including Apilipex. Aripiprazole and other antipsychotic active

INTERACTION WITH OTHER MEDICINAL PRODUCTS AND OTHER FORMS OF INTERACTION: Due to its α_1 -adrenergic receptor antagonism, aripiprazole has the potential to enhance the effect of certain antihypertensive agents. Given the primary CNS effects of aripiprazole, caution should be used when aripiprazole is taken in combination with alcohol or other CNS medicinal products.

Aripiprazole is metabolized by multiple pathways including the CYP2D6 and CYP3A4 enzymes but not CYP1A enzymes. Thus, no dosage adjustment is required for smokers. Upon discontinuation of the CYP2D6 or 3A4 inhibitor, the dosage of Aripiprazole should be increased to the level prior to the initiation of the concomitant therapy. When weak inhibitors of CYP3A4 (e.g., diltiazem or escitalopram) or CYP2D6 are used concomitantly with Aripiprazole, modest increases in aripiprazole concentrations might be expected. Following concomitant administration of carbamazepine, a potent inducer of CYP3A4,

when concomitant administration of Apilipex occurs with carbamazepine. Other potent inducers of CYP3A4 (such as rifampicin, rifabutin, phenytoin, phenobarbital, primidone, efavirenz, nevirapine and St. John's Wort) may be expected to have similar effects and similar dose increases should therefore be applied. Upon discontinuation of potent CYP3A4 inducers, the dosage of Apilipex should be reduced.

PREGNANCY AND LACTATION: Due to insufficient safety information in humans and concerns raised by animal reproductive studies, this medicinal product should

EFFECTS ON ABILITY TO DRIVE AND USE MACHINES: As with other antipsychotics, patients should be cautioned about operating hazardous machines, including motor vehicles, until they are reasonably certain that aripiprazole does not affect them adversely.

• **Other Findings:** Adverse reactions known to be associated with antipsychotic therapy and also reported during treatment with aripiprazole include neuroleptic malignant syndrome, tardive dyskinesia, seizures, cerebrovascular adverse reactions and increased mortality in elderly demented patients, hyperglycemia and diabetes mellitus.

• **Post-Marketing:** The following adverse reactions have been reported during post-marketing surveillance. The frequency of these reactions is considered not known (cannot be estimated from the available data).

Endocrine disorders: Hyperglycaemia, diabetes mellitus, diabetic ketoacidosis, and diabetic hyperosmolar coma.
Metabolism and nutrition disorders: Weight gain, weight decreased, anorexia, and hyponatremia.

Cardiac disorders: QT prolongation, ventricular arrhythmias, sudden unexplained death, cardiac arrest, torsades de pointes, bradycardia.

Hepato-biliary disorders: Jaundice, hepatitis, increased alanine, aminotransferase (ALT), increased aspartate, aminotransferase (AST), increased gamma, glutamyl

Renal and urinary disorders: Urinary incontinence, urinary retention. **Reproductive system and breast disorders:** Priapism.

OVERDOSE: The potentially medically important signs and symptoms observed included lethargy, increased blood pressure, somnolence, tachycardia, nausea, vomiting and diarrhea. In addition, reports of accidental overdose with aripiprazole alone (up to 195 mg) in children have been received with no fatalities. The potentially medically serious signs and symptoms reported included somnolence, transient loss of consciousness, and extrapyramidal symptoms. Management of overdose should consist of supportive care, including monitoring of vital signs and observation of the patient. Management of overdose should consist of supportive care, including monitoring of vital signs and observation of the patient. Management of overdose should consist of supportive care, including monitoring of vital signs and observation of the patient.

with aripiprazole, close medical supervision and monitoring should continue until the patient recovers. Activated charcoal (50 g), administered one hour after aripiprazole, decreased aripiprazole C_{max} by about 41% and AUC by about 51%, suggesting that charcoal may be effective in the treatment of overdose. Although there is no information on the effect on

PHARMACODYNAMIC PROPERTIES: Pharmacotherapeutic group: Other antipsychotics ATC code: N05A12. It has been proposed that aripiprazole's efficacy in schizophrenia and bipolar I disorder is mediated through a combination of partial agonism at dopamine D2 and serotonin 5HT_{1A} receptors and antagonism of serotonin 5HT_{2A} receptors. Aripiprazole exhibited antagonist properties in animal models of dopaminergic hyperactivity and agonist properties in animal models of dopaminergic hypoactivity. Aripiprazole exhibited high

dopamine and serotonin subtypes may explain some of the other clinical effects of aripiprazole. Aripiprazole doses ranging from 0.5 to 30 mg administered once a day to healthy subjects for 2 weeks produced a dose-dependent reduction in the binding of ^{11}C -raclopride, a D₂/D₃ receptor ligand, to the caudate and putamen detected by positron emission tomography.

Distribution: Aripiprazole is widely distributed throughout the body with an apparent volume of distribution of 4.9 l/kg, indicating extensive extravascular distribution. At therapeutic concentrations, aripiprazole and dehydro-aripiprazole are greater than 99% bound to serum proteins, binding primarily to albumin.

Elimination: The mean elimination half-lives for aripiprazole are approximately 75 hours in extensive metabolisers of CYP2D6 and approximately 146 hours in poor metabolisers of CYP2D6

PHARMACOKINETICS IN SPECIAL PATIENT GROUPS: Pediatric population: The pharmacokinetics of aripiprazole and dehydro-aripiprazole in pediatric patients 13 to 17 years of age were similar to those in adults after correcting for the differences in body weights.

Gender: There are no differences in the pharmacokinetics of aripiprazole between healthy male and female subjects nor is there any detectable effect of gender in a population pharmacokinetic analysis in schizophrenic patients.

Hepatic Disease: A single-dose study in subjects with varying degrees of liver cirrhosis (Child-Pugh Classes A, B, and C) did not reveal a significant effect of hepatic impairment on the pharmacokinetics of aripiprazole and dehydro-aripiprazole, but the study included only 3 patients with Class C liver cirrhosis, which is insufficient to draw conclusions on their metabolic capacity.

Keep All Medicines Out of Reach of Children

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